

MEDCORE HOSPITALS

Patient Complaint Resolution Policy

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Document Control

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1. Purpose and Scope

This policy establishes the process for receiving, recording, triaging, investigating, resolving, escalating and closing patient complaints. It applies to Patient Services and departments involved in complaint handling, including Billing, Clinical Operations, Administration and Legal & Compliance.

Control	Standard practice	Responsible role	Escalation trigger
Routine	Follow the approved workflow and document material actions.	Process owner	Exception cannot be resolved.
Verification	Use authorized information and do not guess.	Assigned staff	Identity or source is uncertain.
Traceability	Keep status, owner and next step visible.	Department manager	Case becomes overdue or conflicting.

2. Complaint Intake

Complaints may be received in person, by telephone, in writing or through approved digital channels. The intake record should capture date, time, category, relevant encounter or administrative reference, the patient's stated concern, preferred contact route and any immediate escalation indicator. Staff should record the complaint neutrally and should not convert an allegation into a finding.

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3. Initial Acknowledgement

The receiving employee should acknowledge the concern respectfully, explain that it will be routed through the appropriate review process and avoid promising a particular outcome. If an immediate safety, privacy or other priority issue is identified, the approved escalation should begin without waiting for routine processing.

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4. Categorization

Billing complaints route to Finance, appointment complaints to Scheduling or Patient Services, clinical complaints to Clinical Operations, facility concerns to Administration and privacy concerns to Legal & Compliance. Multi-category complaints may require a coordinating owner.

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5. Investigation Planning

The investigator should define the issue, identify relevant records, establish the questions that need to be answered and determine which staff or functions need to contribute. The investigation record should distinguish patient-reported information from documented evidence.

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6. Evidence Review

Relevant records should be reviewed without altering them merely to support a response. Where evidence is incomplete or conflicting, the uncertainty should be documented. The investigator should avoid unsupported assumptions and should escalate when the issue exceeds the function's authority.

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7. Findings

The case record should state what was reviewed and what the available evidence supports. A patient report, staff explanation and system record may differ; the record should preserve those distinctions. If a matter cannot be resolved from available evidence, that limitation should remain visible.

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8. Resolution

Resolution may include correcting an administrative error, clarifying a billing item, routing a clinical concern, addressing a facility process or taking another authorized action. Staff should not promise refunds, clinical outcomes or remedies outside their authority.

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9. Escalation

Clinical safety concerns should reach Clinical Operations. Privacy matters should reach Legal & Compliance. Billing disputes should reach Finance. Facility safety concerns should reach Administration. Multi-department cases should have a coordinating owner and clear handoffs.

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10. Patient Communication

Communication should be factual, respectful and within the employee's authority. If a review remains open, staff should communicate that status rather than providing an unsupported conclusion. Material communications and commitments should be recorded where required.

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11. Closure

A case should be closed only after the investigation or escalation is complete, required actions have owners and the final communication has occurred where applicable. Overdue cases should be visible to the responsible manager.

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12. Trend Analysis

Management may monitor acknowledgement time, resolution time, overdue cases, complaint volume by category, repeat complaints and closure completeness. Recurring themes should be considered for process, training or communication improvements.

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13. Roles and Governance

Patient Services owns intake coordination and visibility. Department investigators own evidence review and corrective action. Managers own overdue escalation and recurring issue review. The policy owner should review the policy periodically and after material changes.

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14. Scenarios and FAQs

A disputed invoice should be routed to Finance. A privacy complaint should use the Legal & Compliance route. A clinical concern should reach Clinical Operations. A multi-department case should have one coordinating owner. An unresolved case should not be silently closed because the patient stops responding.

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